ADR IDENTIFICATION REPORT

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1. **Case summary**

Female patient, 45 years old, presented with rash, itching, nausea and dizziness.

2. **Medication history**

Amoxicillin: Started 5 days ago (suspected drug)

Paracetamol: Taken as needed (concomitant drug)

3. **Timeline**

Day – 5: Amoxicillin started

Day – 2: Symptoms began

Day –0: Patient reported symptoms

4. **Analysis**

Symptoms began after the drug as started, so the timings fits

Rash and itching are common signs of an allergic reaction to amoxicillin, a penicillin antibiotic

Nausea and dizziness are also known side effect of amoxicillin.

Paracetamol is less likely the cause, since it is taken only as needed and rarely causes rash at normal doses.

No other cause (infection, food, other drugs) is mentioned in the case.

5. **Causality assessment (WHO-UMC)**

Probable: The timing is reasonable, the reaction matches the known profile of amoxicillin, and no other cause is evident.

6. Suspected drug

Amoxicillin

7. **Recommendation**

Stop amoxicillin and monitor the patient

Switch to a non-penicillin antibiotic.

Record penicillin allergy in the patient’s file

Report the ADR to PvPI